



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : مريم هايل مكارى مشرقى
المحافظة : الدقهلية
الحى / المدينة : المنصوره
رقم البطاقة : 29309211200243 رقم التليفون : 01021251888
العنوان : القاهرة حدائق القبة
تاريخ تسجيل الإستمارة : 03-10-2026
العمر (تاريخ الميلاد) : 21-09-1993 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: مريم هاييل مكارى مشرق National ID: 29309211200243	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بمحافظة تخليد الشخصية

مريم
هاثيل مكارى مشرقى
٧ ش ابراهيم مكارى
حائلق القبه - القاهرة

٢٩٣٠٩٢١١٢٠٠٢٤٣

١٩٢٨٢٢٠٨٤

٢٩٣٠٩٢١١٢٠٠٢٤٣

حاصلة على ليسانس ادب
قسم اللغة الفرنسية والادب
اثنى
مينا عبدالملك خلف ولان
البطاقة سارية حتى ٢٠٣٧/١١/١١

٢٠٢٠/١١

متروجة

٥٣٧



المستشفى السعودي الألماني
Saudi German Hospital
Caring like family نرعاكم كأهالينا

Patient Name: MARIEM HABIEL MAKARY MASHIRKY	Patient ID: EG01.0000591479
Report Date: 12/12/2025	Age: 32 Years
Ref. Phy.: Ehab Mossad Abd El Ghany Mohamed Mahmoud Mossad	Sex: F

BOTH BREASTS SONOGRAPHY

Clinical History: Known case of breast cancer on Chemotherapy

FINDINGS:

- Left breast neoplastic-looking lesion is noted at 3 o'clock measuring 5x2 cm with internal suspicious calcifications showing macro lobulated borders. A clipse is noted inside.
- Other suspicious lesions are noted at left breast: at 1 o'clock measuring 1.7x1.3 cm, at 5 o'clock measuring 1.4x0.7 cm, 8 o'clock measuring 5x4 mm, 2 lesions at 12 o'clock measuring 3x3 mm each.
- Surrounding marked subcutaneous tissue edema is noted.
- No suspicious right axillary lymphadenopathy.
- Suspicious left axillary lymphadenopathy; the largest most suspicious measures 8x6 mm with few showing cortical thickenings 5 mm

Conclusion:

- Left breast multi-centric neoplastic-looking lesions as detailed above with surrounding mastitis (possibly mastitis carcinomatosis) and suspicious left axillary lymph nodes (BIRADS 6)
- Unremarkable study of the right breast (BIRADS 1)

Best regards,
Dr. Zeinab Aly, M.D.
Radiology Senior Registrar

Name: مريم هابيل مكارى مشرقى		GUSTAVE ROUSSY	
MR. : 2305			
Detailed Medical Report			
		السيد	
تخية طبية وبعد,,			
		بمء للطلب المقدم من	
بشأن تقرير طبي للمريض			
رقم طبي:		قسم:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الخروج :		تاريخ وساعة الدخول :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient was evaluated for YPT3, YPN2a due to non-PCR findings. The plan included initiation of Kadcyla, placement of a PORT, and pending start of Femara pending Kadcyla approval. Laboratory studies were completed, and the patient was instructed to obtain a postoperative echocardiogram for Femara assessment and to bring the results to the clinic. On a subsequent visit, the patient’s weight was recorded as 93 kg. An echocardiogram performed on 22 March 2025 demonstrated a left ventricular ejection fraction of 66 %. The patient began Cycle 1 (C1) of Kadcyla 260 mg together with Femara, and laboratory results were within acceptable limits. For Cycle 2 (C2), the treatment plan continued with Kadcyla, Femara, and supportive care. Laboratory results were pending, and a dose of Zoladex was scheduled for 10 June. During Cycle 3 (C3), the regimen remained Kadcyla, Femara, and supportive care, with pre-procedure labs pending. Zoladex 10.8 mg subcutaneously was administered. At the time of Cycle 4 (C4), the patient received Zoladex (administered late), Femara, supportive care, and Kadcyla. After laboratory verification, Kadcyla was prepared for Cycle 5 (C5), and Zoladex was scheduled for administration on 20 September. The patient attended a pharmacy visit for Cycle 5 (C5) Kadcyla and Femara. An echocardiogram performed on 18 July 2026 showed an ejection fraction of 64 %. The oncology assessment documented a left-sided breast cancer classified as clinical T3 N2, invasive ductal carcinoma, grade 2, estrogen receptor 7/8, progesterone receptor 7/8, HER2 3+. The treatment history included four cycles of AC, four cycles of taxane, six cycles of carboplatin, and initiation of Phesgo on 20 January 2026. The patient had undergone left mastectomy (YPT3 YPN2) on 7 November, with a recommendation for PORT placement prior to Cycle 6 of Kadcyla 260 mg and continuation of Femara. Zoladex was noted as due on 20 September. The most recent encounter addressed a chief complaint of headache, prompting a brain MRI. The patient remained on Cycle 5 (C5) Kadcyla 260 mg, Femara, and Zoladex with the next Zoladex dose due on 20 September. The patient has completed five cycles of Kadcyla to date.			
تحريرا في ,			
التاريخ :		21-09-2026	
الوقت :		هنا اسامه عيد الرحيم عبد البر	
الطبيب المعالج :			
يعتمد ,			



المستشفى السعودي الألماني
Saudi German Hospital
Caring like family نرعاكم كأهالينا

Patient Name: MARIEM HABIELMAKARY MASHIRKY	Patient ID: EG01.0000591479
Report Date: 12/12/2025	Age:
Ref. Phy.:	Sex: F

BILATERAL DIGITAL MAMMOGRAPHY, CC AND MLO VIEWS
REVEALED:

Clinical History: Under investigations

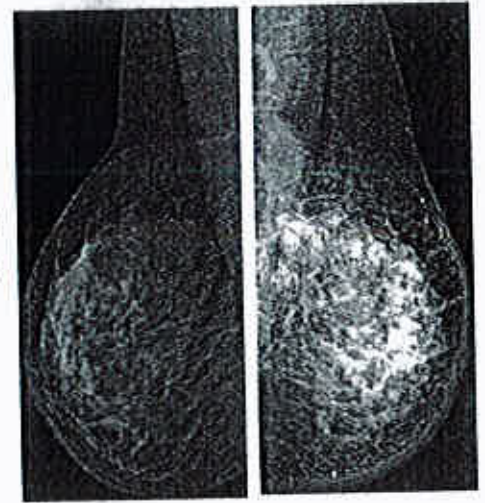
Exposure Dose: 3 u

Findings:

- Predominantly fibroglandular breast (ACR C).
- Left breast upper Outer quadrant shows lesions with opaque margins, microcalcifications and evidence of a clip noted.
- Increased skin thickness reaching 5 mm
- Unremarkable right breast

Opinion:

Left breast upper Outer quadrant lesions with opaque margins, microcalcifications and evidence of a clip noted with increased skin thickness (possible neoplastic process) for further sonographic correlation (M0)



Best regards,
Dr. Zeinab Aly, M.D.
Radiology Senior Registrar



المستشفى السعودي الألماني
Saudi German Hospital
Caring like family نرعاكم كأهالينا

Bill No. : 2CR828940

Order No. : OP2 - 930287

Order Date : 21-Nov-2025 17:04

Patient Name MARIEM HABIEL MAKARY MASHRKY

Age/Gender: 32 Year(s)/Female Room :OPD

Hospital No. : EG01.0000591479

Sample Date :

Doctor Name Mahmoud Shehta Abdelawad Ahmed Mostafa [CAR62]

Test Date : 21-Nov-2025 18:19

CARDIOLOGY (ECHO FULL STUDY)

DESCRIPTION	RESULTS	UNITS	REFERENCE RANGE(S)
LVEDD	4.9	[cm]	Min 3.8 - Max 5.3
LVESD	3.1	[cm]	Min 2.2 - Max 3.5
IVSd	0.7	[cm]	Min 0.6 - Max 1
PWTd	1.0	[cm]	Min 0.6 - Max 1
FS	38	[%]	
LA	3.8	[cm]	< 4 - null
Ao	2.8	[cm]	< 3.7 - null
EF	68	%	Min 54 - Max 74
Examination.			

-- Normal LV internal dimensions with normal LV global systolic function, with estimated EF 68% by m-mode and the same range by 2D eye balling.

-- Normal LV diastolic function.

-- Normal LV wall thickness.

-- No gross RSWMA.

-- Normal mitral valve leaflets morphology, with trivial MR.

-- Normal aortic valve morphology, with no AR, no significant pressure gradient.

-- Normal tricuspid valve morphology, with trivial TR, normal RVSP.

-- Normal pulmonary valve morphology, normal flow, normal gradient.

-- Normal left atrium dimensions, normal aortic root and ascending aorta diameters.

-- Normal right sided chambers, with normal RV systolic function.

-- No intra-cardiac masses or thrombi by TTE.

-- Normal pericardium with no pericardial effusion.

Cairo: 47 Joseph Tito Street, Taha Hussein axis , New Nozha

القاهرة: ٤٧ شارع جوزيف تيتو تقاطع محور طه حسين النزهة الجديدة

Alexandria: Mehwer El Taamear km23 - Alex West Compound

الإسكندرية: الكيلو ٢٣ طريق محور التعمير - كمياوند أليكس
Technologist: [16439] أليكس

NOTE: This report is generated by computer. Any manual correction will make it null and void. **Thank You. We Care!!!**

Echo
[[DOCTYPE:ECHO]]



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : مريم هانييل مكارى مشرفى

Sex:

3- Medical number:

Female

83259836

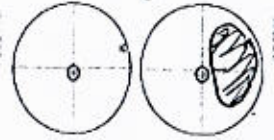
Old Med :

Old Med :

433-2025

2-ID number:

Patient Age : 31 Y



Family history

Family history

Phone

Phone

01289565182

BMI

BMI

REASON OF MDT

REASON OF MDT

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

17/02/2025

Co morbidity

Co Morbidity

Menopausal status

Menopausal Status

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

It breast OQ extensive calcifications with related palpable locally advanced malignant process BIRADS 5

Mets Workup :

PET CT FREE

BX

ER

7/8

PR

7/8

Her 2 Neu

3

SISH

Ki67

BX COMMENT

invasive micropapillary carcinoma

ER COMMENT

PR COMMENT

Her 2 Neu COMMENT

SISH COMMENT

T

CT3

N

3C

M

0 BY PET

Sonomammography

PATIENT HISTORY

ECHO, LABS

CLIP INSERTION

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

NACT ACX4 TAXOTER PHESGO X4 FOLLOWED BY SURGERY ACC TO RESPONSE

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

PATIENT NAME: MAREEM Habeel Makari MASHRKY

GENDER: Female

DATE : 28-Jan-2025 12:09 PM

PATIENT ID: 20225015900

AGE: 31 y

ORDER: Z157706,Z157707

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND**INDICATION:**

- Left breast palpable lump, over the past two years with nipple retraction and bloody discharge.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- The breast is heterogenously dense may obscure small lesions (ACR C).

Left breast:

- Amorphous and plemorphic segmental microcalcifications at the UOQ, measured 10.8x11.4cm, reaching the nipple, with related irregular indistinct masses of high density, associated with skin thickening.
- This was confirmed by Complementary Ultrasound which showed palpable hypoechoic areas of altered parenchyma with echogenic foci of calcification extending from 1 to 4 o'clock, mounting to multiple masses formation, the largest is seen at 4 o'clock invading the overlying skin, measuring 4.5 x 2 cm
- Retro-areolar at 6 o'clock dilated duct with internal echogenic contents.
- Scattered cysts
- Pathologically enlarged axillary level I, II (inter and sub-pectoral), III and supraclavicular lymph nodes, the largest is level I measuring about 2.9 x 1.2 cm.

Right breast:

- No spiculated mass lesions or grouped microcalcifications.
- This was confirmed by Complementary Ultrasound which showed no solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

OPINION:

- Left breast OQ extensive calcifications, with related palpable locally advanced malignant process (BIRADS 5) for histopathological correlation.
- No right breast suspicious findings (BIRADS 2)

PATIENT: MAREEM Habeel Makari MASHRKY, 20225015900, Female, 31 y

Report version 1, 28-Jan-2025 01:22 PM

Page 1 of 2



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16802

مركز بهية الهرم
4 شارع علوية أول شارع الهرم بجوار داري، الجيزة
مستشفى بهية الشيخ زايد
1 - الحي الأول - الشيخ زايد أمام كلية الهندسة



Patient Name : الأستاذة / مريم هابيل مكاري مشرقى
Patient No. : 29309211200243
Path No. : SR-246/25
Sex : Female
Age : 31 Y
Referring Doc :

Request No. : 13001232
Request Date : 10/02/2025
Verification Date : 12/02/2025
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.

Nature of Specimen: Tru-cut biopsy, Slide revision.

Gross Examination:

-One paraffin block labeled by (200275-25) and 3 slides were received.

Microscopic Examination:

- * Histological type: Invasive micropapillary carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
- * DCIS: No.
- * Angioinvasion: No.
- * Perineural invasion: No.
- * Microcalcifications: Undetected.
- * IPT:
- ER: Strong nuclear reaction in 60% of tumor cells (score 7/8).
- PR: Strong nuclear reaction in 50% of tumor cells (score 7/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).

Diagnosis:

Left breast cancer, Tru-cut biopsy, Slide revision:

Invasive micropapillary carcinoma, Grade II.

IPT:

- ER: Positive (score 7/8).
- PR: Positive (score 7/8).
- Her2: Positive (score 3+).

Verified by :

Printing Date : 17/02/2025 10:28:53

Page 1 of 2

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



المستشفى السعودي الألماني
Saudi German Hospital

Caring like family نرعاكم كأهالينا

Patient Name: MARIEM HABIEL MAKARY MASHRKY	Age: 31 Years
Report Date: 3/1/2025	Sex: F
Ref. Phy.: Wesam Reda Farag El Ghamry Reda	Patient ID: EG01:0000591479

- No chest wall invasion .
- Normal MRI appearance of the right breast

CONCLUSION:

- left breast upper and lower outer quadrants (1-5 O'clock) large lobulated rather defined heterogeneous mass lesion measures approximately 8.8x5x6.8 cm in maximum AP, TR and CC span and has the morphological appearance, contrast enhancement pattern and signal behavioral changes as above described" BI-RADS -6"
- For clinical correlation, "dedicated" oncological/surgical consultation, correlation with sono-mammographic, laboratory data (including tumor markers and other specific markers)

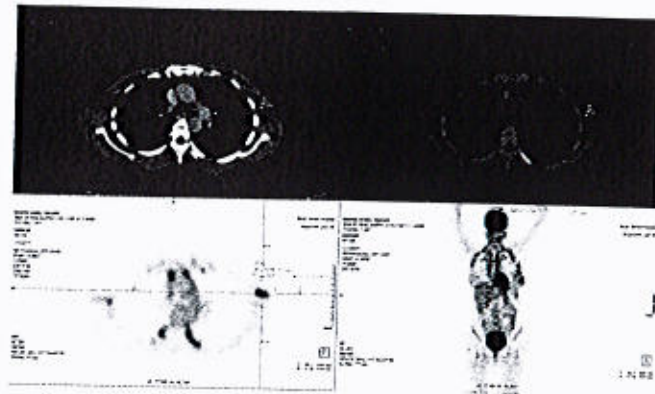
Final Assessment Categories			
Category		Management	Likelihood of cancer
0	Need additional imaging or prior examinations	Recall for additional imaging and/or await prior examinations	n/a
1	Negative	Routine screening	Essentially 0%
2	Benign	Routine screening	Essentially 0%
3	Probably Benign	Short interval follow-up (6 month) or continued	>0 % but ≤ 2%
4	Suspicious	Tissue diagnosis	4a. low suspicion for malignancy (>2% to ≤ 10%) 4b. moderate suspicion for malignancy (>10% to ≤ 50%) 4c. high suspicion for malignancy (>50% to <95%)
5	Highly suggestive of malignancy	Tissue diagnosis	≥95%
6	Known biopsy-proven	Surgical excision when clinical appropriate	n/a

- ACR category a: The breasts are almost entirely fatty.
- ACR category b: There are scattered areas of fibroglandular density.
- ACR category c: The breasts are heterogeneously dense, which may obscure small masses.
- ACR category d: The breasts are extremely dense, which lowers the sensitivity of mammography.

Best regards,

Dr. Ayman A. Khedr, MD
Radiology consultant

Patient Name: MARIEM HABIEL MAKARY	Patient ID: 25.01.01-15:49:30-STD- 1.3.12.2.1107.5.1.4.60063
Study Date: 1/1/2025 3:49:30 PM	Age: 31 Years
Ref. Phy.:	Sex: F



II- HEAD AND NECK:

- No metabolically active pharyngeal or laryngeal lesions.
- No metabolically active cervical lymphadenopathy.

III- CHEST:

- No metabolically active pulmonary nodules, masses, or cavitary lesions.
- No consolidation or collapse. No pleural or pericardial effusion.
- No metabolically active mediastinal lymphadenopathy.

IV- ABDOMEN AND PELVIS

- Enlarged-sized liver with homogenous low parenchymal attenuation. No metabolically active hepatic focal lesions. Metabolically inert lesion in the right hepatic lobe segment II non-enhancing lesion measures 1.3cm, likely cyst. Both kidneys are normal in site, average sized showing smooth regular cortical outline and preserved parenchymal thickness.
- Normal appearance of the spleen, adrenals, pancreas, UB; aorta & IVC.
- No metabolically active pelvic lesions. No ascites or collections seen.
- No metabolically active abdominal or pelvic lymphadenopathy.

V- MUSCULOSKELETAL:

- Spondylo-degenerative changes of the spine.
- No metabolically active lesions could be detected.